
COLDNIL Dry Cough & Congestion Relief Syrup

Name and Strength of Active Substances

Each 5ml contains:

Pseudoephedrine HCl30mg

Dextromethorphan HBr10mg

Preservatives:

Sodium Benzoate0.1% w/v

Potassium Sorbate0.1% w/v

Product Description

Reddish pink colour syrup with cherry flavour and raspberry flavour.

Pharmacodynamics

Pseudoephedrine is a sympathomimetic agent with direct and indirect effects on adrenergic receptors. It has alpha and beta-adrenergic activity and some stimulant effect on the central nervous system. The sympathomimetic effect of pseudoephedrine produces vasoconstriction which in turn relieves nasal congestion.

Dextromethorphan is a cough suppressant.

Pharmacokinetics

Pseudoephedrine is readily and completely absorbed from the gastrointestinal tract and is largely excreted in the urine unchanged. It has an elimination half-life of 5 to 8 hours but its urinary elimination and hence half-life is pH dependent. Pseudoephedrine is rapidly distributed throughout the body, its volume of distribution being 2 to 3 L/kg bodyweight.

Dextromethorphan is well absorbed from the gastrointestinal tract, metabolized in the liver and excreted as both unchanged drug and demethylated metabolites.

Dextromethorphan undergoes rapid and extensive first-pass metabolism in the liver after oral administration. Genetically controlled O-demethylation (CYD2D6) is the main determinant of dextromethorphan pharmacokinetics in human volunteers.

It appears that there are distinct phenotypes for this oxidation process resulting in highly variable pharmacokinetics between subjects. Unmetabolised dextromethorphan, together with the three demethylated morphinan metabolites dextrophan (also known as 3-hydroxy-N-methylmorphinan), 3-hydroxymorphinan and 3-methoxymorphinan have been identified as conjugated products in the urine.

Dextrophan, which also has antitussive action, is the main metabolite. In some individual's metabolism proceeds more slowly and unchanged dextromethorphan predominated in the blood and urine.

Indications

Symptomatic relief of cough and nasal congestion associated with common cold, allergy and other respiratory infections.

Recommended Dose

Adults & children over 12 years: 10ml, three times daily

Children 6-12 years: 5ml, three times daily

Children 2-6 years: 2.5ml, three times daily

Not to be used in children less than 2 years of age. To be used with cautions, and as advised by the physician/pharmacists for children age 2 to 6 years.

Route of Administration

Oral

Contraindications

It is contraindicated in patients with hypersensitivity to pseudoephedrine, dextromethorphan or to any of the excipients, severe hypertension, severe heart disease, taking or have taken Monoamine Oxidase Inhibitors (MAOIs) therapy in the last 2 weeks, taking serotonin reuptake inhibitors (SSRIs), risk of developing respiratory failure and cough associates with asthma or excessive secretions.

Warning and Precautions

WARNING

When used for treatment of cough and cold:

(a) Not to be used in children less than 2 years of age

(b) To be used with caution and doctor's/ pharmacist's advice in children 2 to 6 years of age.

Pseudoephedrine should be used with caution in patients with hypertension, hyperthyroidism, diabetes mellitus, coronary heart disease, ischaemic heart disease, glaucoma, prostatic hypertrophy, severe hepatic or renal dysfunction. Caution is particularly recommended for patients with a history of drug abuse or psychoactive substances. If any of the following signs such as hallucinations and restlessness occur, stop use and seek medical assistance right away. This product should not be taken with any other cough and cold medicine. Avoid alcoholic drink. May cause drowsiness, do not drive while taking this medicine.

Interactions with Other Medicaments

Caution should be exercised with patients receiving other sympathomimetic agents, appetite suppressants or amphetamine-like psychostimulants, as there is a risk of hypertension. Pseudoephedrine may antagonise the pressor effects of antihypertensive agents, such as adrenergic neurone blockers, and severe hypertension may occur in patients receiving beta-blockers. Hypertensive crisis may occur if pseudoephedrine is co-administered with MAOIs. Concomitant use of potent CYP2D6 enzyme inhibitors can increase the dextromethorphan concentrations in the body to levels multifold higher than normal. This increases the patient's risk for toxic effects of dextromethorphan (agitation, confusion, tremor, insomnia, diarrhoea and respiratory depression) and development of serotonin syndrome. Potent CYP2D6 enzyme inhibitors include fluoxetine, paroxetine, quinidine and terbinafine.

Pregnancy & Lactation

Pregnancy

There is limited amount of data on the use of pseudoephedrine in pregnant women. The use of pseudoephedrine during the first trimester of pregnancy has been associated with an increased frequency of gastroschisis (a developmental defect in the abdominal wall with intestinal herniation) and of small intestinal atresia (congenital obstruction of small intestine). Due to the vasoconstrictive properties of pseudoephedrine, it may induce a reduction in uteroplacental circulation. Pseudoephedrine is not recommended in pregnancy.

Lactation

Pseudoephedrine has been detected in human milk with a small percentage of the total maternal dose potentially administered to the breastfed infant. The use of pseudoephedrine should be avoided during breast feeding as lactation may be suppressed, and irritability and disturbed sleep have been reported in breast fed infants. It is not known whether dextromethorphan or its metabolites are excreted in breast milk and therefore use of this product during lactation should be avoided.

Adverse Effects / Undesirable Effects

Dextromethorphan

The following side effects may be associated with the use of dextromethorphan:

Gastrointestinal disorders: vomiting, gastrointestinal disturbances (nausea and diarrhoea).

Nervous system disorders: drowsiness (occasional), dizziness, convulsions.

Psychiatric disorders: excitation, mental confusion.

Respiratory, thoracic and mediastinal disorders: respiratory depression.

Skin and subcutaneous tissue disorders: skin reactions including rash.

Pseudoephedrine

Cardiovascular disorders: tachycardia, palpitations, other cardiac dysrhythmias.

Gastrointestinal disorders: nausea and/or vomiting.

General disorders and administration site conditions: irritability.

Immune system disorders: hypersensitivity reactions, including cross sensitivity that may occur with other sympathomimetics.

Nervous system disorders: headache, tremor, anxiety, restlessness, excitability, insomnia, hallucinations (particularly in children) and paranoid delusions.

Psychiatric disorders: sleep disturbance.

Renal and urinary disorders: difficulty in micturition including urinary retention.

Skin and subcutaneous tissue disorders: skin reactions including rash.

Frequency unknown - Severe skin reactions, including acute generalized exanthematous pustulosis (AGEP).

Vascular disorders: hypertension.

Symptoms and Treatment of Overdose**Dextromethorphan**

It is thought to be of low toxicity, but the effects in overdosage will be potentiated by simultaneous ingestion of alcohol and psychotropic drugs.

Symptoms: These include nausea and vomiting, CNS depression, dizziness, dysarthria (slurred speech), nystagmus, somnolence (drowsiness), excitation, mental confusion, psychotic disorder (psychosis), and respiratory depression, convulsions.

Management: Treatment of overdose should be symptomatic and supportive. Gastric lavage may be of use. Convulsions should be controlled with intravenous diazepam. The specific narcotic antagonist naloxone can be used to reverse the effects of dextromethorphan.

Information for children

Naloxone has been used successfully to reverse central or peripheral opioid effects of dextromethorphan in children (0.01mg/kg body weight).

Pseudoephedrine

Symptoms: Symptoms of overdosage include abdominal discomfort, excitation, confusion, hallucinations, ataxia, irritability, restlessness, palpitations, hypertension, difficulty in micturition and thirst.

Management: In severe overdosage gastric lavage and aspiration should be performed. Symptomatic and supportive measures should be undertaken, particularly with regard to the cardiovascular and respiratory systems. Chlorpromazine may be used to control marked excitement and hallucinations. Severe hypertension may need to be treated with an alpha-adrenoreceptor blocking drug, such as phentolamine. A beta-blocker may be required to control cardiac arrhythmias.

Effects on Ability to Drive and Use Machine

This medicine can impair cognitive function and can affect a patient's ability to drive safely. Patients should be advised not to drive or operate machinery if affected.

Storage Conditions

Keep container tightly closed. Store below 30°C. Protect from light.

Dosage Forms and Packaging Available

Oral liquid (60ml, 90ml and 100ml per bottle)

Registration Number:

MAL

Manufactured by & Product Registration Holder

KCK Pharmaceutical Industries Sdn. Bhd.

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Date of Revision:

6 September 2022